BASELINE → TREATMENT → FOLLOW-UP

When checks happen

Milestones show order, not equal time intervals or booked appointments. The treating team requests tests, reviews results and confirms treatment. FBC, renal and liver profile before every 28-day cycle; assess hands/feet, mouth, infection and symptoms. Perform regular ECG monitoring and LVEF assessment according to risk and cumulative exposure. LVEF assessment is mandatory before each further PLD administration that exceeds lifetime cumulative anthracycline 450 mg/m²; risk can occur below this threshold.

Check	Before starting	Early treatment	Ongoing treatment	Dose change / pause	Next decision
Blood / kidney / liver	Baseline FBC, kidney and liver tests	FBC, kidney and liver tests before each 28-day cycle	FBC, kidney and liver tests before each 28-day cycle	Recheck recovery and dose before restarting	Follow unresolved abnormalities
Symptoms / medicines	Review symptoms, medicines and previous treatment	Review effects before each dose	Continue symptom and medicine reviews	Report problems; do not change or restart doses yourself	Agree recovery and next treatment plan
Heart / ECG / echo	Baseline ECG and echo/MUGA, regular ECG and risk-based LVEF checks throughout treatment; mandatory LVEF before further administration beyond the source cumulative threshold.	Review symptoms and indicated tests	Continue the risk- and treatment-specific plan	Urgent assessment for chest pain, fainting or breathlessness	Agree follow-up from exposure and results
Response / course	Confirm cancer-specific treatment aim	Typically 6 cycles or continue according to response and tolerance	Review benefit, symptoms and the agreed scan plan	Reassess if treatment is held or symptoms change	Agree continuation, change or completion
Bone health / other care	Review previous/ongoing treatments and fracture risk	No automatic chemotherapy-cycle DXA	Keep any separate endocrine/ fracture-risk plan	Review if clinical risks change	Continue separately indicated follow-up

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00205 v8 · official regimen · EMA Caelyx pegylated liposomal product information

THE WHOLE TREATMENT JOURNEY

Pegylated liposomal doxorubicin · every 4 weeks: treatment and checks

Source-defined advanced breast, ovarian, endometrial cancer or soft-tissue sarcoma

Pegylated liposomal doxorubicin on Day 1 every 28 days, typically for six cycles or until progression or unacceptable toxicity. The team confirms the intended duration and reviews benefit.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	Early treatment	Ongoing treatment	Dose change / pause	Next decision
Treatment plan NCCP 00205	Confirm indication, goal and current prescription	Typically 6 cycles or continue according to response and tolerance	Continue according to response, tolerance and prescribed course	Team review before restart or dose change	Agree completion or next treatment
Blood / kidney / liver NCCP 00205 · SMPC	Baseline FBC, kidney and liver tests	FBC, kidney and liver tests before each 28-day cycle	FBC, kidney and liver tests before each 28-day cycle	Recheck recovery and dose before restarting	Follow unresolved abnormalities
Symptoms / medicines NCCP 00205 · SMPC	Review symptoms, medicines and previous treatment	Review effects before each dose	Continue symptom and medicine reviews	Report problems; do not change or restart doses yourself	Agree recovery and next treatment plan
Heart / ECG / echo NCCP 00205 · SMPC	Baseline ECG and echo/MUGA, regular ECG and risk-based LVEF checks throughout treatment; mandatory LVEF before further administration beyond the source cumulative threshold.	Review symptoms and indicated tests	Continue the risk- and treatment-specific plan	Urgent assessment for chest pain, fainting or breathlessness	Agree follow-up from exposure and results
Response / course NCCP 00205 · SMPC	Confirm cancer-specific treatment aim	Typically 6 cycles or continue according to response and tolerance	Review benefit, symptoms and the agreed scan plan	Reassess if treatment is held or symptoms change	Agree continuation, change or completion
Bone health / other care NCCP 00205 · SMPC	Review previous/ongoing treatments and fracture risk	No automatic chemotherapy-cycle DXA	Keep any separate endocrine/ fracture-risk plan	Review if clinical risks change	Continue separately indicated follow-up

What each check is for

1

Treatment and safety review

Before each planned treatment or four-week review for olaparib

FBC, renal and liver profile before every 28-day cycle; assess hands/feet, mouth, infection and symptoms. Perform regular ECG monitoring and LVEF assessment according to risk and cumulative exposure.

Ask / act: The treating team confirms results and the plan; this map does not authorise a dose.

Sources: NCCP 00205 · SMPC

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Benefit and continuation

At agreed clinical review points

In a study in metastatic breast cancer, this liposomal formulation controlled cancer about as well as conventional doxorubicin and caused fewer heart problems, but more hand–foot and mouth problems. It can still affect the heart, and personal benefit is uncertain.

Ask / act: Discuss benefit, toxicity and priorities before continuing or changing treatment.

Sources: NCCP 00205 · RETROSPECTIVE-TRIAL

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Sources checked 2026-10-25 · Source reconciled; independent clinical, pharmacy and patient/PPI review pending · SACTCheck v0.84.21